A/PROF CHAMARA BASNAYAKE
MBBS, PhD, FRACP
Gastroenterologist & Endoscopist
PN 421926PK

FOCUS GASTROENTEROLOGY

29 April 2026

Dr Anthony Mariampillai
Doctors On Broadway

Dear Dr Mariampillai,

Re: Dr Lalith Perera
DOB: 22/06/1949 Phone: 0412 494 839
Address: 16 Grattan Street, CARLTON, VIC, 3053
Email: lalithperera008@gmail.com
Medicare: 3239388593 / 1

Summary

- **Gastroscopy findings of a hiatus hernia and a Schatzki ring.**
- **Colonoscopy revealing significant diverticulosis but no polyps or malignancy.**
- **Negative biopsies for Helicobacter pylori, coeliac disease, and microscopic colitis.**
- **Management of Metformin-induced diarrhoea via fibre supplementation.**
- **Surveillance colonoscopy recommended in five years.**
- **No action required by you.**

I had the pleasure of conducting a review with Lalith to discuss the results of his recent gastroscopy and colonoscopy. These investigations were primarily performed to investigate symptoms of diarrhoea and a family history of colorectal malignancy.

I am pleased to report that the colonoscopy did not reveal any evidence of malignancy or colonic polyps. There was significant diverticulosis noted throughout the colon, which made the procedure somewhat technically challenging, but the remainder of the examination was unremarkable. Histological analysis of the colonic biopsies excluded microscopic colitis as a cause for his symptoms.

The gastroscopy identified a hiatus hernia and a Schatzki ring, which is a structural narrowing in the distal oesophagus usually associated with chronic gastro-oesophageal reflux. Biopsies of the stomach were negative for Helicobacter pylori, and small bowel biopsies excluded coeliac disease.

Impression and Plan Lalith continues to experience approximately three loose bowel movements per day, which he attributes to his use of Metformin (Diabex). He currently denies any symptoms of dysphagia, heartburn, or regurgitation. Given he is asymptomatic, I do not believe the Schatzki ring requires surgical intervention or the initiation of a proton pump inhibitor at this time.

To address the diarrhoea, I have recommended the introduction of a daily fibre supplement, such as psyllium husk or Metamucil. This should improve stool consistency. Furthermore, increased fibre intake may reduce the risk of future diverticulitis and offer potential metabolic benefits. We discussed the role of probiotics; however, I advised him that the clinical evidence is currently modest and inconsistent, and I have instead encouraged a diet high in natural fibre. Due to his family history, I have recommended a surveillance

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colonoscopy in five years. Thank you for referring Lalith.

Yours sincerely,

A/Prof Chamara Basnayake
Gastroenterologist

Investigation Results:

PERERA, LALITH : SVH 1120623, EPW 3119554, SVHP 1120623

DOB: 22/06/1949 Age: 76 years

16 Grattan Street, Carlton 3053

Results from 15/04/2026 ..... Episode# 26P276047 Test Hist (Histology - Routine)

FINAL ANATOMICAL PATHOLOGY AAnat

Lab #: 2026B009605 **CLINICAL NOTES** Gas = antrum erosion. Duo = N. - Random colon=N. Gastric polyp? Fundic. **MACROSCOPIC DESCRIPTION** 1. Specimen labelled "Gastric polyp". Two tan polypoid fragments 1 x 1 x 1 mm 2 x 2 x 2 mm. Processed whole. A1. 2. Specimen labelled "Gastric". Two mucosal fragments measuring 2-3 mm. A1. 3. Specimen labelled "Duodenum". Three mucosal fragments measuring 2-3 mm. A1. 4. Specimen labelled "Random Colon bx". Three mucosal fragments measuring 2-3 mm. A1. **MICROSCOPIC DESCRIPTION And DIAGNOSIS 1. Gastric polyp - Benign cystic fundic gland polyp. No dysplasia is seen. 2. Gastric mucosal biopsy - Mild chronic gastritis with antral type glands showing preserved architecture. A few scattered lymphocytes and plasma cells. No Helicobacter organisms seen. 3. Duodenal mucosal biopsy - Normal duodenum with preserved villous height to crypt length ratio. A normal complement of lymphocytes and plasma cells. No neutrophilic infiltrate is seen. 4. Random colonic mucosal biopsy - Normal colonic mucosa with preserved glandular architecture. A normal complement of lymphocytes and plasma cells. The subepithelial collagen plate is of normal thickness. No neutrophilic infiltrate is seen.** Pathologist: A/Professor J. Slavin

All appointments and enquires
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Endoscopy locations
Epworth Freemasons Day Proc. Centre
320 Victoria Pde, East Melbourne, 3002
Hobson Healthcare Sydenham
566 Melton Hwy, Sydenham, 3037
St Vincent's Private Hospital
59 Victoria Pde, Fitzroy, 3065